TB-11 Eye Trauma

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | Reconciled October 17, 2026

Core principle: Treat life threats first. Irrigate chemical exposure immediately, but never apply pressure, irrigate, manipulate, instill drops, or remove an object when a penetrating or open-globe injury is suspected.

Field flow

1. Chemical or thermal? For chemical exposure: start immediate copious water or saline irrigation; remove a contact lens only if easy. Do not neutralize.	2. Open globe? No pressure, manipulation, irrigation, drops, or object removal. Stabilize any protruding object and apply a rigid shield.	3. Blunt / superficial? Check visual acuity and red flags. Irrigate loose debris only if penetration or globe rupture is not suspected.	4. Protect + transport Pain/nausea control, minimize Valsalva, serial vision assessment, and early notification for vision-threatening injury.

Provider-level actions

Level	Approved actions
EMT	Treat ABCs and associated trauma. Start immediate chemical/thermal irrigation. For suspected open globe, stabilize without pressure and use a rigid shield. Assess and document visual acuity in each eye separately when this does not delay care.
AEMT	All EMT care plus clinically indicated vascular access and authorized analgesia or antiemetic care. Do not delay irrigation, protection, or time-sensitive transport for access.
Paramedic	All prior care plus advanced airway, analgesia, antiemetic, monitoring, Medical Control, and destination decisions. Do not use ketamine for suspected penetrating or open-globe eye trauma unless Medical Control expressly authorizes it.

Vision-threatening findings

Urgent transport / early notification: decreased or lost vision; irregular or teardrop pupil; hyphema; visible penetration or extrusion; impaled object; marked proptosis or rapidly increasing swelling; severe pain; significant chemical burn; high-velocity mechanism; or inability to examine the eye.

Focused assessment

• Record mechanism, time, laterality, chemical or product, contact-lens use, protective eyewear, high-velocity metal/glass exposure, and associated head or facial trauma.

• Inspect without pressure: pupils, bleeding, hyphema, corneal clouding, foreign body, lid injury, proptosis, peri-orbital swelling, and visible tissue or fluid.

• Check extraocular movement or diplopia only gently and only when a globe injury is not suspected. Never force the eyelids open.

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CHEMICAL, OPEN-GLOBE, AND TRANSPORT SAFEGUARDS

Chemical and thermal eye injury

• Begin copious water or normal-saline irrigation immediately and continue during transport as needed. Water is preferable to waiting for saline or a specialized irrigating fluid.

• Remove contact lenses only when easily possible without delaying irrigation. Irrigate from the inner corner outward and prevent runoff into the unaffected eye or onto responders.

• Brush away dry chemical before irrigation when safe. Do not attempt acid-base neutralization. Use the Chemical / HazMat pathway and Poison Help guidance for unusual or water-reactive agents.

• For thermal injury, gently cool or irrigate with room-temperature water or saline. Do not use ice, ointment, or a pressure dressing.

Open globe, blunt trauma, and foreign body

Situation	Approved response
Suspected open globe	Do not apply pressure, irrigate, manipulate, instill medication, or remove an object. Stabilize a protruding object with bulky material around it without pressure, then apply a rigid shield.
Blunt trauma	Use a rigid shield when globe injury is possible. Check for hyphema, irregular pupil, vision change, diplopia, restricted movement, proptosis, and associated facial/head injury.
Loose debris	Irrigate only loose superficial debris when penetration or globe rupture is not suspected. Do not remove embedded material or use cotton swabs, magnets, or instruments.
Medication	Use approved analgesia and antiemetic care; minimize coughing, vomiting, agitation, and Valsalva. Topical ophthalmic anesthetic is unavailable unless separately stocked and adopted or ordered by Medical Control.

Never patch either eye. For suspected open-globe injury, protect the affected eye with a rigid shield without pressure. A pressure patch can worsen an open-globe injury.

Documentation and handoff

• Document baseline and repeat vision, pupils, laterality, mechanism, product/exposure time, irrigation start and duration, protection, medications, response, consultation, and destination rationale.

• This direct source PDF replaces the prior source and matches Claiborne-approved app content reconciled October 17, 2026.